

BEHAVIOURAL DISORDERS IN CHILDREN

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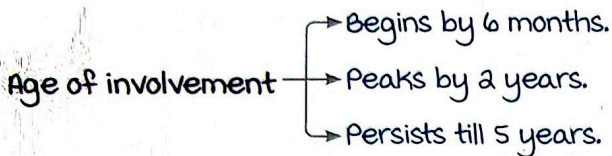
Breath holding spells, Bruxism and Pica

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BREATH HOLDING SPELLS

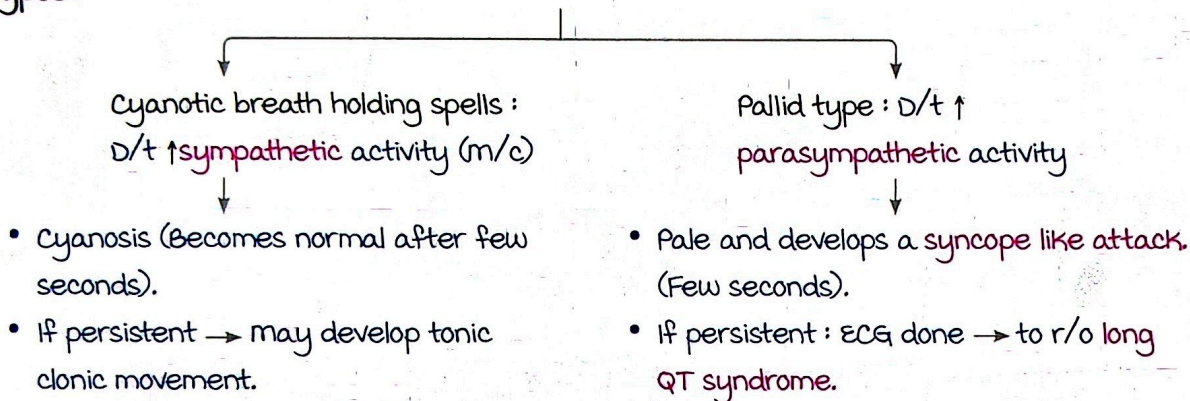
Reflex event.



Sequence of events :

Provocative factors (Anger, pain) → Baby cries → Holds breath in **full expiration**.

Types :



management :

- Reassure parents (Benign condition with spontaneous resolution).
- During tonic clonic episode : Turn the child sideways (prevents aspiration).
- Avoid picking the child up during event (causes sudden ↓ cerebral perfusion).
- Evaluate and treat for **iron deficiency anaemia**.
- **Atropine** : For long duration pallid spells.

BRUXISM

Teeth grinding.

Onset and occurrence :

- Age : > **5 years** (<5 yrs : normal).
- Occurs during sleep.
- Daytime bruxism : Associated with **anxiety disorders**.

Feedback

----- Active space -----

Long term problems :

- Dental malocclusion.
- Jaw pain.

Treatment → <5 years → No treatment required
 → >5 years → Behavioural therapy.

PICA**Definition**

Consumption of inedible/non nutritive substances (chalk, mud, paint) :

- For **atleast 1 month**.
- Inappropriate for **development or cultural practices**.

Age : < 5 years.

Risk factors :

- malnutrition
- **Iron deficiency anemia**
- Low socioeconomic status
- Psychosocial stress.
- Developmental delay (Cerebral palsy).

Complications :

- Lead poisoning (From paint).
- Parasitic infestations (From mud).

management :

- Behavioural therapy.
- Deworming (Albendazole Stat).
- Low dose iron supplements (0.5 to 1 mg/kg/day)

Thumb sucking, Temper tantrums and Tics

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THUMB SUCKING

Self soothing behaviour for a child.

Age of involvement → Begins after 6 months
 → Peak : 18 to 21 months
 → Normal till 4 years

management : → <4 years : Reassurance
 → >4 years (Indicates **emotional stress/ social insecurity**)
 • Behavioural modifications (Positive reinforcement like praise).

Note : Avoid negative reinforcement (like applying oil on thumb).

TEMPER TANTRUMS

Attention seeking behavior.

Age of involvement → Begins at 18-36 months.
→ Persists till 6 years.

Presentation :

Physical/emotional challenges → Crying/kicking/pushing/head banging.

management :

- Reassure & advice parents to remain calm during episode.
- Child to be left alone in a place where unlikely to get harmed.
- **Distraction** (Taking the child away from that place).
- **Time out technique** (Leaving the child alone for sometime to settle down).

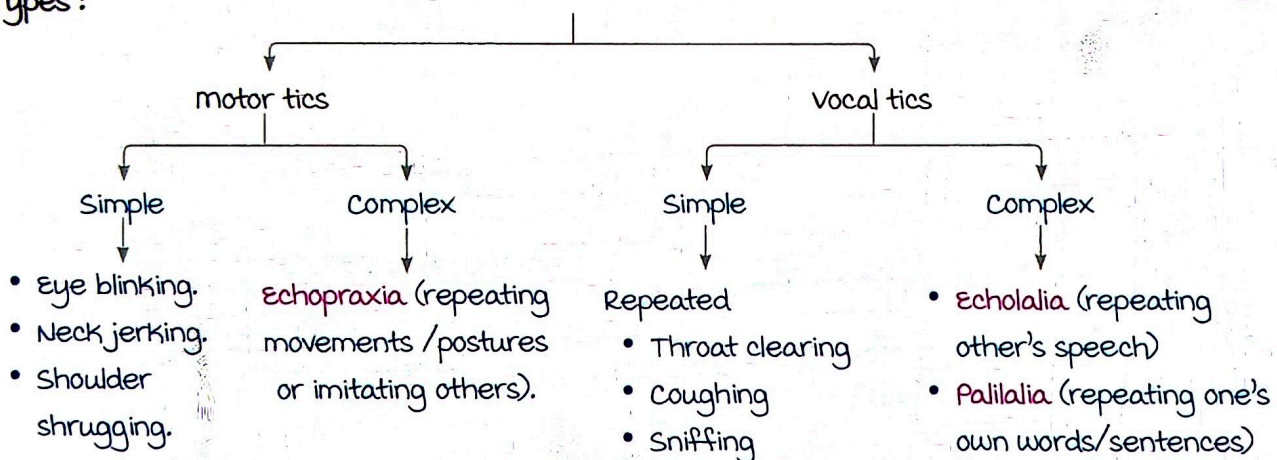
TICS

Abrupt onset of fast, **involuntary**, **paroxysmal**, non rhythmic, repetitive motor/verbal manifestations.

Age of involvement → Begins by 4-6 years.
→ Peak : 10 to 12 years.
→ Persists till 18 to 20 years

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Types :



Tourette's syndrome :

- Age of onset < 18 years.
- Motor and vocal tics for ≥ 1 year.

management :

- Behavioural therapy.
- Long lasting tics : Neuroleptics (**Haloperidol**, Clonidine).

Feedback

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Growth and Development

----- Active space -----

Nocturnal Enuresis

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Definition :

Nearly complete evacuation of bladder at >5 yrs (age of bladder maturation) : At a wrong place & time for ≥ 2 times a month for ≥ 3 months.

Incidence : males $>$ Females.

Types :

Based on etiology

Primary (m/c)

- Present since birth.
- Etiology (Functional disorder) :
 - Delayed bladder maturation.
 - Decreased ADH secretion at night.

Secondary

- Previously dry at night for at least preceding 6 months
- Etiology (Organic) :
 - UTI
 - Stress
 - DI/Dm
 - Bowel bladder dysfunction.

Based on symptomatology

monosymptomatic (m/c) :

Passing urine at night time.

Polysymptomatic :

Passing urine at night time + Lower urinary tract symptoms/LUTS (Hesitancy, urgency & dribbling of urine)

m/c variety : Primary monosymptomatic nocturnal enuresis.

management :

motivational + Alarm therapy : Highest success rate (60-70%).

Non Pharmacological	Pharmacological
• Reassurance. • Behavioural modifications : - voiding before going to bed, - Restrict fluids in the evening - Avoid caffeine drinks (tea, coffee and soda). • motivational therapy : verbal praising, gifts : if dry at night. • Alarm therapy : Conditioning a child to sensation of full bladder. - Sensors put in underwear $\rightarrow$ Detects wetness & awakens child. - Simple alarms set to the time of usual bed wetting.	• Short term : Oral desmopressin. • Long term usage : Oxybutynin/Tolterodine ($\downarrow$ uninhibited bladder contractions). Imipramine not used d/t adverse cardiovascular side effects.